



MOTHER BABY UNIT PROCEDURE	
Baby Boarders in the Mother Baby Unit	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mother Baby Unit (MBU)
This document should be read in conjunction with the Disclaimer .	

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Aim

The purpose of this guideline is to ensure that staff are aware of the care requirements for babies boarding with their mothers in the Mother and Baby Unit (MBU).

Key Points

Mothers are admitted as inpatients to the MBU and their babies are registered onto the hospital's patient administration system as boarders. The mother is responsible for the baby's care during hospitalisation and should be supported to undertake this. However, despite the baby's status as a boarder, mental health clinicians on the MBU do have a duty of care and responsibility for the baby while a boarder on the MBU.

Process

Referrals to the MBU

- The MBU Clinical Nurse Manager (CNM) triages all referrals to the MBU. The triage process includes gathering information on the current physical health of the baby, particularly if they are unwell.
- All referral information is documented in PSOLIS as a service event for the mother. The referral is added to PSOLIS, but baby information is not added.
- A baby may be admitted as a boarder to the MBU up to 12 months of age if the baby is non-ambulant. Babies less than 12 months who are ambulant will not be admitted.
- The Clinical Nurse Consultant (CNC) will inform the Child Health Nurses (CHN) of the admission to MBU by emailing the [Special Referral Form](#) to Child Adolescent Health Service (CACH) Birth Notifications global email box - BirthNotificationsCDIS.CACH@health.wa.gov.au.
- [Child Health MBU Handover Flowchart](#)
- If the baby is currently unwell with a potential infectious condition, the management of the baby will be discussed with the Infection Prevention and Management team, and appropriate quarantine periods implemented as per the [WNHS Standard and Transmission-Based Precautions Policy](#) and [CAHS Transmissible Diseases Index](#).

Physical Health Requirements of Baby Boarders

On admission to the ward:

- The baby's weight is recorded by nursing staff on admission and then weekly using the MBU Baby Assessment Form, the Girl/Boy Growth Charts (if required), and the MBU Weight Chart.
- New medical issues identified by the parents or MBU staff will be reported to the MBU Resident Medical Officer (RMO) or Registrar. A temperature check will be recorded and reported to the Medical Officer to escalate/discuss with the paediatric team, if required. A set of physiological observations will be completed as required.
- The MBU RMO will consult with the Perth Children's Hospital (PCH) for advice and assistance when required.
- Routine Baby Health checks will be facilitated by the MBU either by the mother (if well enough), family or discussion with the relevant CHN.

Paediatric health concerns for the baby such as failure to thrive; prematurity-associated health issues or other non-acute physical health conditions need to be referred to appropriate paediatric services if required during their MBU inpatient stay.

Acute physical health concerns or clinical deteriorations require escalation as below:

- Between 0800–1630hrs (Monday to Friday) the MBU RMO or Registrar or medical officer will determine if the baby should be taken to the Perth Children's Hospital Emergency Department (ED) or to the family General Practitioner (GP) for assessment.
- Between 1630-0800hrs, weekends and public holidays MBU staff will escalate care to the the Hospital Clinical Nurse Midwife Specialist and advise the on call Medical Officer/Registrar or Psychiatrist.
- If the baby is for review in the PCH ED,
 - If available and appropriate, family members in the first instance will attend ED with mother and baby. If clinically indicated, MBU staff will escort the mother and baby to the ED and provide clinically relevant documentation as required to facilitate review. Neonatal or paediatric code blue will be managed as per the [WNHS Recognising and Responding to Acute Clinical Deterioration \(Physiological and Mental Health\)](#).

Feeding and Sleeping

- On admission, nursing staff will discuss feeding and sleeping patterns with the mother.
- During the first 48 hrs nursing staff will monitor and identify if there are any mother crafting issues that require a specified intervention by the multi-disciplinary team.
- Throughout the admission, staff will provide support and intervention as required.
- All care provided by staff should be communicated in a prompt and timely manner back to the mother using MR208.15 Baby Feed and Sleep Chart – MBU.
- The mother will be informed that care will be provided in line with [WA Health Mandatory Policy MP 0106/19 Safe Infant Sleeping Policy](#).
- The mother will be provided with written information on safe infant sleeping.
- All mothers are provided with education and support to record the baby's sleeping and feeding routine on the MR208.15 Baby Feed and Sleep Chart – MBU during their admission.
- Concerns regarding feeding routines will be escalated to the Medical Officer and documented in the baby's healthcare record.
- Referrals may be made using E-referral to Lactation Consultants at the KEMH Breastfeeding Centre or the PCH feeding clinic, depending on the baby's needs.
- If there are concerns regarding the baby's growth, feeding, or reflux, the baby may be referred to the PCH or the patient's own Paediatrician, regular family GP, or local GP clinic.

Medication administration

- On admission, all currently prescribed or over the counter medications the parents have been administering to the baby will be handed to the nursing staff and treated as the Patient's Own Medications (POM) as per the [WNHS Medicines Management Framework Patient's Own Medications Policy](#).

This will be explained on admission to the mother and partner.

- New medications prescribed while the baby is in hospital, will be charted on the medication chart, and will comply with the WA State-wide Medicines Formulary Guideline.
- MBU staff will administer prescribed medications to the baby and use the opportunity to provide patient/partner coaching when appropriate
- Non-imprest items will require dispensing at a community pharmacy. These items will be sourced by the patient's partner/support person and subsequently handed over to MBU staff for storage and administration.
- Imprest items supplied by the pharmacy are limited to:
 - Paracetamol liquid, oral
 - Nystatin, oral drops
 - Chloramphenicol, ophthalmic
 - Hydrocortisone, topical.
- The Women and Newborn Health Service (WNHS) MBU Pharmacist will provide medication chart review for baby boarders.

Documentation

- Care provided to the baby will be documented onto a hard copy of [Mother and Baby Assessment form MR209](#) and [Integrated Progress Notes MR250](#).
- On discharge, all documentation for the baby is filed in the baby's healthcare record.
- Details about the baby's time on the ward will be documented in the 'additional information' section of the mothers Notification and Clinical Summary (NaCS) and a copy sent to the nominated GP.
- Child health nurse handover form completed and sent to relevant CHN on discharge.

Compliance and Evaluation

The MBU Clinical Nurse Manager will monitor compliance with this procedure via healthcare record auditing and routine review of clinical incident reporting.

Related policies and procedures

Related policies

[National Principles for Child Safe Organisations](#)
[NMHS Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
[WA Health MP 0106/19 Safe Infant Sleeping Policy](#)
[WA Health MP 0077/18 Statewide Medicines Formulary Policy](#)
[WNHS Medicines Management Framework Patient's Own Medications Policy](#)
[WNHS Patient Identification and Procedure Matching Policy](#)
[WNHS Patient Identification Procedure](#)
[WNHS Recognising and Responding to Acute Clinical Deterioration \(Physiological and Mental Health\) Policy](#)
[WNHS Standard and Transmission-Based Precautions Policy](#)

Related WNHS procedures and guidelines

[Women's and Perinatal Mental Health Referral and Management Guideline](#)
[WNHS Patient Identification Procedure](#)

Useful resources (including related forms)

[MR208.15 Baby Feed and Sleep Chart – MBU](#)

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NSQHS Standards Applicable:	 Std 3: Preventing and Controlling Healthcare Associated Infection  Std 4: Medication Safety		 Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration		
Chief Psychiatrist's Standard for Clinical Care	Assessment Care planning Consumer and Carer Involvement in Individual Care				
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Version History

Version Number	Date	Summary
1.0	May 2020	Clinical Practice Guideline Title Baby Boarders: Care in the Mother and Baby Unit
2.0	01/8/2021	Baby Boarders: Care in the MBU guideline rescinded Title change: WNHS procedure: Baby Boarder MBU Procedure. Amendments made to all sections -Refer to guideline/procedure. Changes to align with new formatting

3.0	August 2025	Three yearly review. Amendments include: Minor formatting and rewording. Stable links added. Added: • WNHS Standard and Transmission-Based Precautions Policy and CAHS Transmissible Diseases Index. • Link to referral form to CAHS • Advise the Hospital Clinical Nurse Midwife Specialist • National Principles for Child Safe Organisations Changed: • set of physiological observations to temperature check and observations recorded as required. • Infant History to MBU Baby Assessment Form • Perth Children's Hospital ED instead of ED • WNHS Recognising and Responding to Acute Clinical Deterioration (Physiological and Mental Health). Title and hyperlink amended • GP to regular family GP • medical to healthcare record • removal of National Standards for Mental Health Service and replaced with Chief Psychiatrist's Standards for Clinical Care to align with NMHS Mental Health guideline documents Removed • All care provided to the baby boarder will be communicated to the mother during bedside handovers- communicated in multiple ways.
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